

Public Health Emergency Operations Center
"COUSP DRC"

MVE B-17 Incident Management System

Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°082/MVE B_04/08/2026

Affected provinces (5)	HAUT-UELE, ITURI, NORTH KIVU, SUD-KIVU & TSHOPO
Affected Health Zones (51)	- Haut-Uélé (5/13): Boma Mangbetu, Isiro, Pawa, Rungu and Wamba - Ituri (28/36): Aru, Aungba, Bambu, Bunia, Damas, Drodoro, Gethy, Kilo, Komanda, Lita, Logo, Mambasa, Mangala, Mongbwalu, Nizi, Nyankunde, Rimba, Rwampara, Tchomia, Kambala, Nia-Nia, Fataki, Mandima, Lolwa, Boga, Ariwara, Mahagi and Adja. - North Kivu (12/34): Beni, Butembo, Goma, Kalunguta, Katwa, Kyondo, Oicha, Vuhovi, Mabalako, Masereka, Musienene and Lubero. - South Kivu (1/34): Miti-Murhesa - Tshopo (5/23): Kabondo, Lubunga, Makiso-Kisangani, Mangobo and Wanie-Rukula
Reporting date	August 4, 2026
Publication date	August 5, 2026

KEY INDICATORS — CUMULATED AS OF 04 AUGUST 2026

CONFIRMED CASES — 5 PROVINCES 3,973 +99 new cases (Ituri 85 · N-Kivu 10 · Haut-Uélé 4)	CONFIRMED DEATHS · LEATHITIS 1,801 · (45.3%) +52 deaths today (Ituri 47 · N-Kivu 4 · H-Uélé 1)	SUSPECTED CASES OF THE DAY 270 Ituri 182 · N-Kivu 66 · Haut-Uélé 21 · Tshopo ND · South Kivu 1
PATIENTS IN ISOLATION 674 Ituri 468 · N-Kivu 186 · Haut-Uélé ND · Tshopo 3 · South Kivu 17	CURED — CUMULATIVE 776 +27 recovered (Ituri 25 · N-Kivu 2)	NATIONAL CONTACT TRACKING 75.3% 13,393 / 17,781 views · Ituri 77.4% · N-Kivu 74.0% · H-Uélé 49.9% · Tshopo ND · South Kivu 100%

1. Epidemiological Highlights

- Epidemiological situation:** No new health zones have been affected in the last 24 hours.

The epidemic remains confined to 51 out of 140 health zones, spread across five provinces: Ituri (28/36), North Kivu (12/34), Haut-Uélé (5/13), Tshopo (5/23), and South Kivu (1/34). North Kivu has 74 affected health zones out of 684, and Haut-Uélé has 22 out of 213.

- Case Update:** Over the past 24 hours, 99 new confirmed cases, 52 confirmed deaths, and 27 recoveries have been recorded, bringing the totals to 3,973 confirmed cases, 1,801 deaths, and 776 recoveries, representing a case fatality rate of 45.3%. Today's deaths include 37 community deaths confirmed by post-mortem testing (71.2%) and 15 deaths occurring in treatment centers. However, the national death toll has increased by only 50: the provincial total for North Kivu published in the previous edition exceeded the sum of its health zones by two, a discrepancy that has been resolved in this edition.

o **Ituri:** 85 new confirmed cases were reported by the laboratory, notified to Bunia (25), Nizi (21), Rwampara (18), Lita (9), Mongbwalu (5), Fataki (5) and Mangala (2), as well as 47 deaths, 35 of which were community deaths (74.5%).

Twelve deaths in the treatment centers were distributed among Rwampara (3), Nia-Nia (3), Bambu (2), Bunia (1), Mongbwalu (1), Nizi (1), and Nyankunde (1). Six (6) positive retests were also returned, bringing the total number of positive cases for the day to 91.

o **North Kivu**: 10 new confirmed cases in Katwa (5), Butembo (3), Kalunguta (1), and Musienene (1), and 4 deaths, all recorded in Katwa. o **Haut-Uélé** : 4

new confirmed cases in Isiro, including 3 living cases and 1 death confirmed by swab. o **Tshopo** : No new confirmed cases or deaths were reported. The provincial total remains at 7 confirmed cases.

5 deaths and 1 recovery, representing a case fatality rate of 71.4%.

o **South Kivu** : The provincial total remains at 3 confirmed cases, including 1 death and 2 recoveries, in addition to 2 cases probable. The province has gone 56 days without a new confirmed case since June 10, 2026.

- **Analysis**: Ituri remains the epicenter with 87.1% of cumulative cases and 85.9% of daily confirmations, and its case fatality rate (42.3%) remains significantly lower than that of North Kivu (67.0%), which now accounts for only 10.1% of new cases. This difference likely reflects later detection in North Kivu, where the in-CTE mortality rate reaches 36%. Nationally, 37 of the 52 daily deaths (71.2%) occurred in the community, compared to 63.6% the previous day. Contact tracing has declined to 75.3% from 78.4% the previous day, well below the operational threshold of 95%. The decline is driven by Haut-Uélé, which fell to 49.9% after the listing of 131 new contacts, and by Ituri (77.4%), where Mangala (846 unseen contacts) and Bunia (810) alone account for 67.0% of the 2,475 unseen contacts. The alert investigation rate also declined to 71.2% nationally.

EPIDEMIOLOGICAL AND OPERATIONAL CONTEXT

Declared on May 15, 2026, the 17th Ebola virus disease (EVD) outbreak caused by the Bundibugyo ebolavirus continues to evolve within a complex humanitarian and security context. Initially confined to the Mongbwalu health zone (Ituri), it The outbreak has spread to five provinces (Ituri, North Kivu, South Kivu, Haut-Uélé, and Tshopo), now affecting 51 health zones. Population movements, insecurity, artisanal mining activities, and cross-border trade with Uganda and South Sudan continue to facilitate transmission. Multisectoral coordination remains mobilized to strengthen surveillance, intensify response efforts, and limit the spread of the epidemic.



Figure 1 — Chronology of events of the Ebola-Bundibugyo virus disease epidemic

2. Descriptive Epidemiological Analysis

2.1 Distribution by province and active households

TABLE 1 — DISTRIBUTION OF CONFIRMED CASES AND DEATHS BY PROVINCE

Province	Confirmed cases	Death	Lethality	Affected health zones	New cases (24 h)
Ituri	3,460	1,464	42.3%	28 / 36 (77.8%)	85
North Kivu	436	292	67.0%	12 / 34 (35.3%)	10
Haut-Uélé	67	39	58.2%	5/13 (38.5%)	4
Tshopo	7	5	71.4%	5/23 (21.7%)	0
South Kivu	3	1	33.3%	1/34 (2.9%)	0
Total	3,973	1801	45.3%	51 out of 140	99

Case fatality rate recalculated from daily totals. No new health zones have been affected in the last 24 hours.

Ituri province accounts for 87.1% of confirmed cases (n = 3,460) and 81.3% of deaths (n = 1,464), with a case fatality rate of 42.3%. The health zones of Bunia (950 cases), Rwampara (692), Mongbwali (558), Nizi (431), Lita (151), and Nyankunde (115) represent 83.7% of the cases reported at the provincial level and 72.9% of the national total. North Kivu, the second-largest outbreak hotspot, saw a 2.3% increase in cases in a single day and maintains a significantly higher case fatality rate than Ituri.

2.2 Distribution by health zone

TABLE 2 — CONFIRMED CASES AND DEATHS BY PROVINCE AND HEALTH ZONE (CUMULATIVE AND CURRENT SITUATION)

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	3,460	1,464	42.3%	85	35	12	47
Adja	11	1	9.1%	0	0	0	0
Aru	5	5	100.0%	0	0	0	0
Ariwara	7	2	28.6%	0	0	0	0
Aungba	10	5	50.0%	0	0	0	0
Bambo	69	17	24.6%	0	0	2	2
Boga	1	1	100.0%	0	0	0	0
Bunia	950	289	30.4%	25	11	1	12
Damascus	18	8	44.4%	0	0	0	0
Drodro	11	6	54.5%	0	0	0	0
Fataki	40	20	50.0%	5	3	0	3
Gethy	1	0	0.0%	0	0	0	0
Kambala	2	0	0.0%	0	0	0	0
Kilo	29	11	37.9%	0	0	0	0
Komanda	38	27	71.1%	0	0	0	0
Lita	151	89	58.9%	9	3	0	3
Logo	10	5	50.0%	0	0	0	0
Lolwa	7	2	28.6%	0	0	0	0
Mahagi	4	1	25.0%	0	0	0	0
Mambasa	10	6	60.0%	0	0	0	0
Mandima	16	12	75.0%	0	0	0	0
Mangala	108	67	62.0%	2	0	0	0

Mongbwalu	558	268	48.0%	5	3	1	4
Nia-Nia	112	66	58.9%	0	0	3	3
Nizi	431	211	49.0%	21	9	1	10
Nyankunde	115	35	30.4%	0	0	1	1
Rimba	9	4	44.4%	0	0	0	0
Rwampara	692	282	40.8%	18	6	3	9
Tchomia	45	24	53.3%	0	0	0	0
North Kivu	436	292	67.0%	10	1	3	4
Blessed	64	49	76.6%	0	0	0	0
Butembo	92	69	75.0%	3	0	0	0
Goma	1	0	0.0%	0	0	0	0
Kalunguta	8	4	50.0%	1	0	0	0
Katwa	200	129	64.5%	5	1	3	4
Kyondo	13	10	76.9%	0	0	0	0
Lubero	1	1	100.0%	0	0	0	0
Mabalako	1	0	0.0%	0	0	0	0
Masereka	5	1	20.0%	0	0	0	0
Musienene	45	25	55.6%	1	0	0	0
Oicha	4	3	75.0%	0	0	0	0
Vuhovi	2	1	50.0%	0	0	0	0
Haut-Uélé	67	39	58.2%	4	1	0	1
Boma Mangbetu	13	5	38.5%	0	0	0	0
Isiro	25	14	56.0%	4	1	0	1
Pawa	12	11	91.7%	0	0	0	0
Rungu	1	0	0.0%	0	0	0	0
Wamba	16	9	56.2%	0	0	0	0
Tshopo	7	5	71.4%	0	0	0	0
Kabondo	1	1	100.0%	0	0	0	0
Lubunga	1	0	0.0%	0	0	0	0
Makiso-Kisangani	3	3	100.0%	0	0	0	0
Mangobo	1	1	100.0%	0	0	0	0
Wanie-Rukula	1	0	0.0%	0	0	0	0
South Kivu	3	1	33.3%	0	0	0	0
Miti-Murhesa	3	1	33.3%	0	0	0	0
Total	3,973	1801	45.3%	99	37	15	52

2.3 Mapping of confirmed cases

The spatial distribution confirms transmission is primarily concentrated in Ituri, particularly in the health zones of Bunia, Rwampara, Mongbwalu, Nizi, Lita, and Nyankunde. Transmission continues in the already affected areas without any new geographical expansion in the last 24 hours. Seven health zones in Ituri reported confirmed cases on August 4.

compared to eleven the day before, but the daily volume increases from 58 to 85 cases.

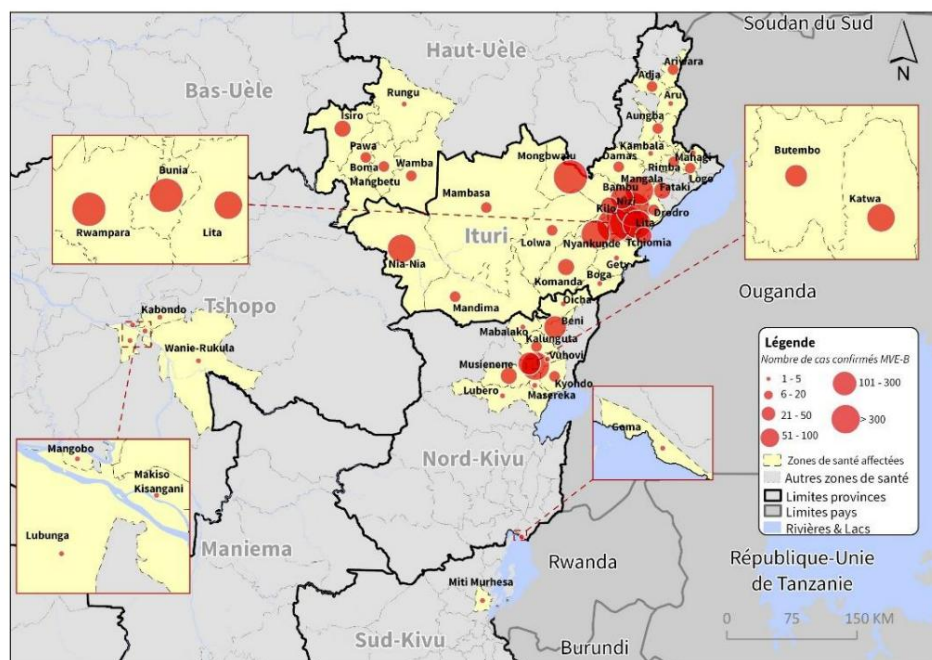


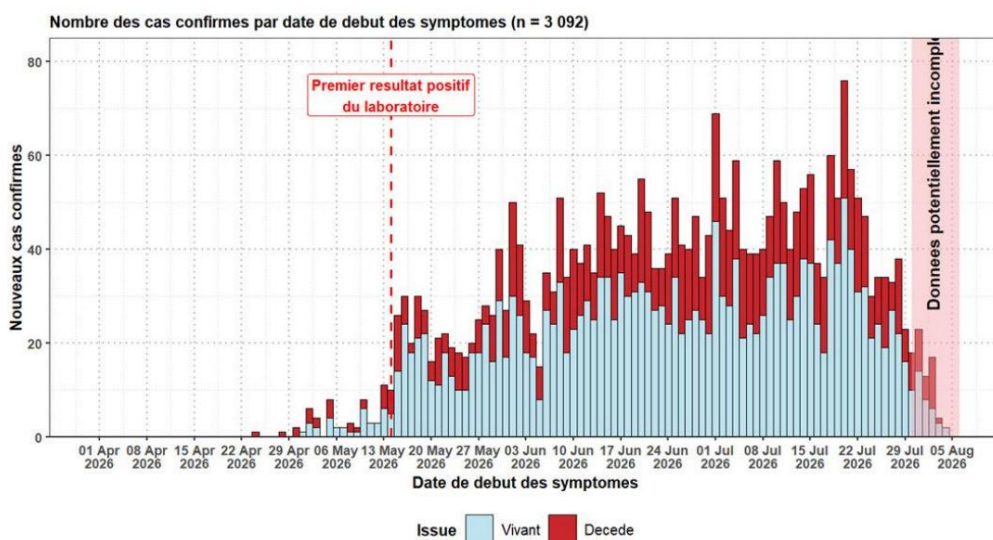
FIGURE 2 — SPATIAL DISTRIBUTION OF HEALTH ZONES THAT HAVE REPORTED CONFIRMED CASES

Transmission continues in already affected health zones, necessitating simultaneous intensive interventions at the epicenter and preparedness activities in neighboring provinces.

2.4 Temporal analysis — epidemic curves

The epidemic curve shows sustained transmission during weeks S26 to S31, with fluctuations related to reporting delays and the gradual consolidation of data. In North Kivu, week S31 was the highest since the start of the epidemic with 101 reported cases, following 82 in week S30.

FIGURE 3 — EPIDEMIC CURVE BY WEEK OF NOTIFICATION



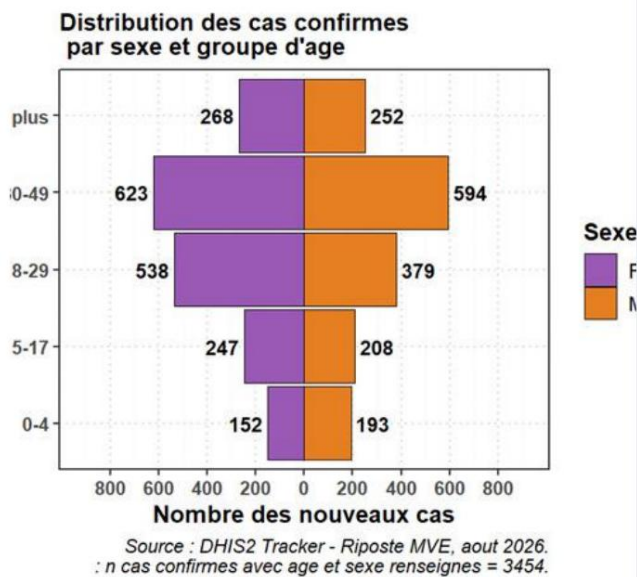
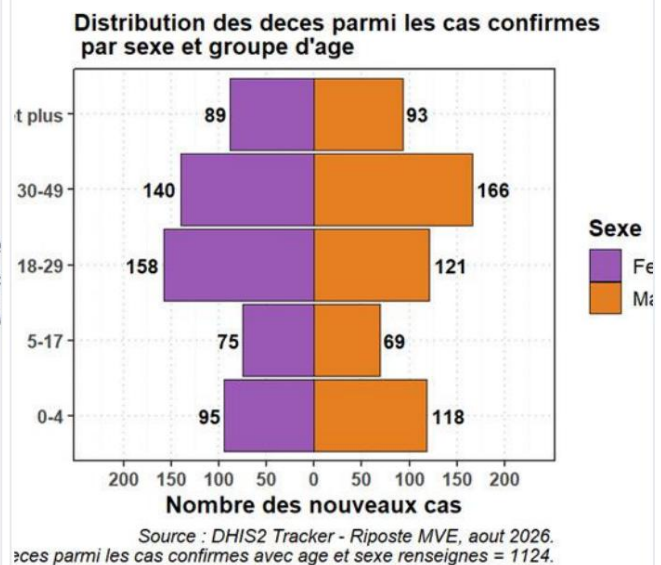
Source : DHIS2 Tracker - Riposte MVE, août 2026. Données incluses : cas confirmés avec statut vital et date de début des symptômes renseignés, n = 3 092.

FIGURE 4 EPIDEMIC CURVE BY DATE OF SYMPTOM ONSET (LINEAR LIST DHIS2)

The epidemic remains in a phase of sustained transmission, with a high level of virus circulation. Recent fluctuations should be interpreted with caution due to reporting delays and the gradual consolidation of data.

2.5 Demographic Characteristics

The age pyramid shows a predominance of working-age adults (18–50 years). Minors under 18 and people over 50 are proportionally less affected.

FIGURE 5 — CONFIRMED CASES BY SEX AND AGE GROUP**FIGURE 6 — DEATHS BY SEX AND AGE GROUP (AMONG CASES) (CONFIRMED)**

The distribution of cases by sex remains balanced. In North Kivu, 229 confirmed cases are male and 207 are female, with the 18–29 and 30–49 age groups accounting for 232 of the 436 cases (53.2%). The predominance of cases is among adults of this age group. active reflects more frequent community and occupational exposures, while the proportion of pediatric cases testifies to the persistence of intrafamilial transmission.

3 Monitoring, testing and reporting

3.1 Alert Management and Sources

TABLE 3 — MANAGEMENT OF EPIDEMIOLOGICAL ALERTS

Indicator	Ituri	North Kivu Haut-Uélé	Tshopo	South Kivu	Total
Report (D-1)	7	156	0	ND	163
New alerts received	562	582	22	ND	1,172
Total alerts for the day	569	738	22	ND	1,335
Alerts investigated	279	570	21	ND	876
Investigation rate (24 h)	49.0%	77.2%	95.5%	ND	100.0%
Alerts confirmed — live	113	40	19	ND	173
Alerts confirmed — deceased (comm.)	61	26	2	ND	89
Total suspected cases for today	179	66	21	ND	267

In total, 1,335 alerts were managed (including 1,172 new ones) and 876 investigated within 24 hours (71.2%), resulting in 267 confirmed suspected cases (173 living and 89 community deaths). North Kivu accounts for the majority of these with 738 alerts, of which 570 were investigated (77.2%): Beni alone accounts for 245. Ituri has 569 alerts but is investigating only 279 (49.0%). Haut-Uélé verifies 21 of its 22 alerts within 24 hours (95.5%) — the only unverified alert, in the Ibambi State Health District (Pawa), remains untraceable. South Kivu is investigating its 6 alerts (Kadutu 4, Miti-Murhesa 2), of which only one has been validated. Tshopo has not reported any alerts dated August 4th.

3.2 Laboratory: tests and positivity

Ÿ **Ituri** : 251 samples received (162 samples from suspected cases, 57 swabs and 32 re-samples) and 247 tested, including 91 positives — 50 living, 35 deceased and 6 re-samples, i.e. a positivity of 36.8%.

Ÿ **North Kivu**: 98 samples received and tested (33 in Beni and 65 in Butembo), of which 10 came back positive, including 5 in Katwa including 4 deaths, 3 in Butembo, 1 in Kalunguta and 1 in Musienene.

Ÿ **Haut-Uélé**: 4 samples tested positive, all in Isiro (3 live and 1 death). Installation and commissioning of the solar system of the MVE diagnostic laboratory, funded by the WHO.

Ÿ **Tshopo**: No results were reported for the day; 12 samples from suspected cases were received and unpacked from Bas-Uélé; biologists were briefed on Ebola virus disease diagnosis, and Red Cross members received training on biosecurity, swab collection, and sample transport. The provincial total remains at 7 positive cases.

Ÿ **South Kivu**: 16 samples received (Katana 11, Kaziba 3, Kadutu 1 and Nyangezi 1), all negative. The provincial total stands at 3 positive results out of 299 samples analyzed (1.0%); 94% of samples are analyzed within one day of collection and 16 tests remain pending.

3.3 Contact tracking

TABLE 4 — CONTACT TRACKING OF CONFIRMED CASES

Province	Contacts under follow-up	Contacts viewed	tracking rate
Ituri	10,942	8,467	77.4%
North Kivu*	6,143	4,551	74.1%
Haut-Uélé**	641	320	49.9%
South Kivu	55	55	100.0%
Tshopo***	ND	ND	ND
Total	17,781	13,393	75.3%

Contact tracing has fallen to 75.3% nationally, compared to 78.4% the previous day, and remains well below the operational threshold of 95%. Ituri is monitoring 8,467 contacts out of 10,942 (77.4%) and has identified 447 new contacts, but two areas account for 66.9% of the 2,475 contacts not seen: Mangala (846 not seen, 47.9% follow-up) and Bunia (810 not seen, 41.6%), followed by Nizi (344) and Fataki (122). The areas of Bambu, Damas, Kambala, Kilo, and Lolwa remain at zero listed contacts despite having cumulative cases. North Kivu has identified 4,551 contacts seen out of 6,143 (74.1%), with 318 lost to follow-up and 1,095 never seen. Beni (64%), Katwa (73%), and Butembo (77%) are dragging down the provincial performance. Haut-Uélé saw a drop to 320 contacts seen out of 641 (49.9%) after 131 new contacts were listed in Isiro, while Boma Mangbetu fell to 25.0% and Isiro to 43.7%. South Kivu saw all 55 of its contacts, 44 of whom were in cantonment. Tshopo did not report any data dated August 4.

3.4 Entry points and control points (PoE/PoC)

3.4.1 — Crossing point alerts (24 h)

Ituri — 9 alerts notified

Nine alerts were reported at border crossings in the province: eight living individuals and one deceased person. Six living individuals, all detected at the Mudzi Bala Point of Control (AS Nzere, Bunia) among travelers arriving from Iga-Barrière, were isolated, validated, and referred to the CTE in Bunia; two were ruled out after secondary screening, at the Makoko 2 Point of Control (Mambasa) and at Bunia Airport. The deceased person, a Sudanese national in transit from Ombayi (Ariwara) to Uganda for burial, was reported and validated but not swabbed. The province's indicator table shows eight validated alerts and seven referred.

North Kivu — 1 alert notified

A live alert was notified to the Mukulia PoC and invalidated after thorough investigation. The completeness of the reporting was 88.9% (16 PoE/PoC out of 18) and 97.5% of travelers were screened.

Tshopo — not reported

The PoE/PoC pillar did not present data dated August 4; the province's indicators are reported as ND.

Haut-Uélé — 0 alerts notified

No alerts were issued to the province's activated PoE/PoCs. Eight of the ten activated PoE/PoCs reported data (80%), and 10,417 travelers were screened, with the Magambe PoC alone accounting for 2,780 (27%). The Dusu and Apodo PoCs did not submit their morning data. The province has only 10 of its 33 priority PoE/PoCs activated, and no alerts have been issued to them since the start of the outbreak.

3.4.2 Monitoring indicators for PoE/PoC

TABLE 5 — MONITORING OF INDICATORS AT POINTS OF ENTRY AND CHECKPOINTS (POE/POC)

Indicators	Ituri North Kivu		South Kivu	Tshopo Haut-Uélé		Overall
Proportion of PoC enabled and PoE enhanced	100% (60/60)	100% (18/18)	100% (9/9)	21.1% (4/19)	30.3% (10/33)	72.7%
Number of people who have switched to PoE/PoC	97,733	ND	0	ND	10,417	108,150
% of travelers screened	96.2%	97.5%	0	ND	100.0%	96.6%
% of travelers who washed their hands	96.2%	ND	0	ND	100.0%	96.6%
% of travelers made aware	99.1%	ND	0	ND	100.0%	99.2%
Number of alerts notified	9	1	0	ND	0	10
Number of validated live alerts	8	0	0	ND	0	8
Lifeless bodies intercepted today	1	0	0	ND	0	1
Problem contacts intercepted today	0	0	0	ND	0	0
% of validated live alerts referred to CT/CTE	87.5% (7/8)	0% (0/1)	0	ND	0	87.5% (7/8)
% of the lifeless bodies swabbed	ND	0	0	ND	0	ND
Positive results from referred suspected cases	0	0	0	ND	0	0

4 Mental Health and Psychosocial Support (MHPSS)

TABLE 7 — KEY SMSPS INDICATORS

SMSPS Key Indicators	Ituri	North Kivu
New confirmed cases that have received interventions	18	ND
Confirmed cases are being monitored.	119	ND
New suspected cases	30	ND
Suspected cases are being monitored	101	ND
Laboratory results announced (including positive ones)	68 (18)	ND
Children separated — nursery / community	0 / 52	ND
CTE support staff	167	ND
Frontline Personnel (PPL)	5	ND
Members of households and communities	75	ND
EDS supported (SMSPS)	14	ND

5 Frontline Personnel (PPL) assigned

TABLE 8 — FRONTLINE STAFF (FLS) INFECTED WITH EVD IN ITURI FLS = frontline health

workers and providers. Table not updated as of August 4: the provincial presentation does not publish a breakdown by health zone and reports a cumulative total of 134 infected FLS, less than the 140 documented here (see DQA).

Health Zone	Confirmed cases	Healed	In hospital	Death
Bunia	35	19	1	14
Rwampara	33	25	1	7
Nia-Nia	22	5	12	5
Mongbwalu	17	7	1	9
Nyankunde	14	11	1	2
Lita	4	1	2	1
Mangala	4	2	1	1
Nizi	3	0	0	3
Logo	3	0	3	0
Bamboo	2	2	0	0
Kilo	2	1	1	0
Damascus	1	0	0	1
Total — case fatality rate 30.7%	140	71	23	43

6 Pillar-Based Response Activities

COORDINATION

ÿ Ituri: data validation meeting between the surveillance and laboratory pillars and the planning section; active call to health zones for data collection and visit to Rwampara to assess the functioning of the alert cell.

CONTINUITY OF ESSENTIAL SERVICES (CSSE)

ÿ Ituri: 29 ambulances, 24 of which were available; 44 trips completed, all successful, for 46 patients evacuated; four ambulances remained out of service (Bambu, call center, Mongbwalu, and Rwampara), and two refused to

ŷ North Kivu: participation in the coordination meeting of the Musienene SZ, facilitation of the training of surveillance focal points for this zone (surveillance, PCI, CREC, medical and psychosocial care) and support for database managers; harmonization of alert and contact databases. ŷ Haut-Uélé: morning meetings held (2/2, 100%) and completion of CREC zonal reporting from 4 out of 5 health zones (80%: Isiro, Pawa, Wamba and Boma); of the seven recommendations followed, two are in progress — support for the release of recovered cases from Wamba and Isiro and equipping of the CTEs in the affected areas — and five remain unfulfilled, including the creation of sub-coordinations in Wamba, Pawa and Watsa and the daily centralization of presentations from the 13 pillars. ŷ Tshopo: the three recommendations of August 3 are partially implemented: the list of contacts of the death that occurred on a boat that left Kisangani between July 19 and 21 has been shared, contact has been initiated with the advisor to the Minister of the Interior to extract the high-risk contacts of the lifeless body brought to Kabondo, and the statements of needs for PPE and laboratory inputs have been signed but not yet shared with the partners.	Transfers were recorded at the call center and in Bambu. Other health zones did not report their movements. ŷ North Kivu: the presentation of August 4 does not include a section on continuity of essential services; no data is reported for the day.
LOGISTICS ŷ Ituri: Official launch of the Rwankole CTE by the provincial governor, with a capacity of 120 beds. Today's deliveries: IPC and EDS kits to Isiro, IPC/WASH and EDS kits to Mongbwalu, IPC kits (boots and surgical gowns) to the Bunia CTE CME, IPC/WASH kits to Lita, hand sanitizer to the national coordination office, and a Starlink kit to the Africa CDC coordination office; preparations are underway for Nia-Nia and the Kilo Health Zone. 49 vehicles and 10 ambulances have been allocated to the pillars and health zones, including 15 for IPC/EDS and 8 for surveillance; 100 liters of fuel have been supplied to the provincial SGI. ŷ North Kivu: receipt of a donation of WHO IPC kits from Bunia, awaiting unloading; delivery of nutritional inputs (Plumpy'Nut, F75 and F100) to the health zones of Kyondo, Butembo and Masereka and 4 mattresses to Kalunguta as compensation for those incinerated during the decontamination of a household; continuation of construction work on the Musienene treatment center (MSF) and Beni (MSF Netherlands); support for the Vuhovi ZS in defining the triage circuit at	PCI / WASH ŷ Ituri: 27 rings opened around cases and 57 sites identified, all decontaminated (100%), with no gaps at 48 hours; 26 health centers monitored and supported (100%), 4 health centers decontaminated in Aru, and 27 means of transport decontaminated (Mongbwalu 14, Nizi 8, Lita 5). 55 households of cases decontaminated within 48 hours out of the expected 82 (67%), with insufficient decontamination teams and PPE, and a lack of replacement kits identified as the causes. 112 community health workers trained (Nia-Nia 100, Lita 12), and Category 2 infection prevention and control/sanitation (IPC/WASH) kits provided to four health centers in Bunia (Bankoko, Aéro, Simbiliambo, and Rwambuzi) with support from PPSSP/UNICEF. Completeness: 9 health zones out of 10. The cumulative number of contaminated PPLs is reported to be 134 out of 2,884 exposed (4.6%), with no new cases today. ŷ North Kivu: none of the 12 households with confirmed cases have been decontaminated; 4 healthcare facilities housing cases have been decontaminated, including 3 in Katwa (Irangya Health Center, Vighole Health Center, MSF) and 1 in Musienene (La Renaissance Hospital); incomplete IPC kits were provided to 2 healthcare facilities in Musienene; 36 healthcare facilities were monitored and supported (Oicha 28, Katwa 5, Musienene 2, Kyondo 1) and 158 service providers briefed during supervision sessions

isolation, its makeshift CTE requiring rehabilitation. ŷ Tshopo: five liaison vehicles mobilized, including three provided by WHO and one vehicle from the MTN-CTP coordination, plus two vehicles for CDC Africa and two for CDC Atlanta; allocation of an ambulance to the Bafwasende health zone; water supply at PK 23; harmonization sessions with the community around the erection site, with work scheduled to begin on August 5.	formative (Musienene 134, Katwa 18, Kyondo 6). One community death benefited from an EDS in Musienene and 4 community deaths were the subject of a swab. Completeness: 4 ZS out of 11. ŷ Tshopo: IPC assessment of 8 health facilities, with scores ranging from 22.6% (CM Renouveau 2) to 59.7% (CT Mpox), and follow-up of 7 health facilities with 22 briefed service providers; household decontamination of the Baego case (Bafwasende) and briefing of 5 decontamination workers from this health area. The cumulative total reached 32 out of 81 health facilities assessed (36.6%), with only one scoring above 80%; 19 out of 81 health facilities received IPC kits (23%) and 5 have a triage system in place (6%).
COMMUNICATION (RCCE / CREC) ŷ Ituri: 4,112 people reached in their homes during 14 rounds of visits conducted by 42 Community Health Workers (CHWs), 3,603 during 28 educational talks, and 4,558 during mass awareness campaigns (Aru, Gety, Komanda), and 17 people engaged during 2 advocacy meetings (Aru, Tchomia); 162 community alerts reported (137 living, 25 deceased) in Nizi, Rwampara, and Tchomia. Of 314 feedback reports, 190 were recorded and 124 clarified (39.4%). 106 CHWs trained in Mongbwalu and Lita (WHO, Africa CDC); 2 radio programs and 6 radio spots produced with FHI 360 and REMED, 36 community radio stations involved, and 15 IEC materials distributed. A traditional chief and 17 members of his family have joined the EBOPEP campaign in Rwampara, where the admission of patients to the CTE CME was also disrupted by the anger of hygiene workers demanding payment of their bonuses. ŷ North Kivu: The national Minister of Agriculture was involved in communicating preventive measures to approximately 900 people in Kyondo; a community dialogue was held with 76 community leaders, led by the mayor of Bulengera, on managing incidents involving response teams in Kyaghala (Katwa), and a second dialogue was held with 65 community leaders and local leaders in the Kyambogho health district. Regarding confirmed cases, reluctance to decontaminate a household was overcome in Butsili (Beni), and contact tracing was supported in Musienene. 145 cases were covered, and 34 household heads were affected; 3 community incidents were resolved, and 16 rumors were clarified. ŷ Tshopo: Pillar meeting, situation analysis and deployment of support teams to health zones (12 participants); market awareness-raising	HOLISTIC CARE (HCC) ŷ Ituri: 25 new recoveries; 468 patients in isolation (285 confirmed, 183 suspected) out of 833 beds, representing 56.2% occupancy, and 60 severe or critical cases, with 81 admissions and 86 discharges. Three Ebola Treatment Centers (ETCs) are over capacity for confirmed cases — Nizi (233%), ISTM (147%) and Bambu (140%) — and one for suspected cases (Logo, 150%). Twelve confirmed deaths and six suspected deaths occurred at the Ebola Treatment Center (ETC). The Rwankole ETC was officially inaugurated by the military governor; a strike and demonstration by ETC staff took place in the morning. Nutritional support: eight patients were placed on a special diet, and 1,018 hot meals were served, including 745 to contacts. A registered dietitian (PPL) was among the new confirmed cases at the ETC. ŷ North Kivu: 11 confirmed cases and 43 suspected cases were admitted, with 2 recoveries (Katwa and Butembo), 6 deaths, and 58 discharges as non-cases; 186 patients remained in isolation at the end of the day (26 confirmed, 160 suspected). The provincial cumulative total reached 187 cases admitted to the Ebola Treatment Center (ETC), 67 deaths, and 18 lost to follow-up, for an ETC mortality rate of 36%. A confirmed patient's refusal to transfer to the Musienene ETC resulted in her escape. An evaluation of the Musienene ETC pathway, rehabilitated by MSF, is underway, and the evaluation mission continues in Katwa, Matanda, and at the University Hospital Center (CUG). Regarding nutrition, 17 confirmed cases, 59 suspected cases, 110 frontline healthcare providers, and 26 caregivers received hot meals, and 109 patient households received free food distributions. ŷ Tshopo: a new admission — a 13-year-old suspected case at the CT of HC Kisangani, arriving from Nia-Nia on July 27 — for 3 patients in isolation at the end of the day (1 confirmed, 2 suspected) out of 20 beds (15.0%) and no discharges. The cumulative number of confirmed hospitalized cases stands at 4, including

central Kisangani by the youth of URNSC (1,100 participants), support for the training of IT staff from the five health zones of the city (92 participants) and of actors from the river police and shipowners within the framework of the "River without Ebola" project (85 participants); 17 community alerts collected, continuation of the dissemination of spots and communication around the cases of Kongakonga and Kabondo. ŷ South Kivu: the community-based door-to-door communication strategy is in its 10th day; none of the 57,638 households targeted in Ibanda (41,389) and Nyangezi (16,249) were visited on August 4, no sick people were referred to a health center and no visitors from the affected areas were identified. ŷ Haut-Uélé: 1,271 people reached in homes across 458 households, 23 through educational talks, and 963 through mass awareness campaigns, including 156 MARFRO members in Wamba and 38 ECZ members in Pawa; briefing of the Pawa CREC zonal team (10 members) to intensify interventions. 4 out of 14 radio stations broadcast in Isiro (RTK, Racom Nava, Congo Espoir Uélé, and RTNC), and 55 IEC tools were distributed, bringing the total to 285. Of 7 feedback points collected, 5 were clarified. The posters received are only available in French and Swahili, which limits their reach in local languages.	Two deaths, one recovery, and one ongoing treatment as of day 6, stable condition. Experimental treatments continue for two high-risk contacts in Madula (Wanie-Rukula), one of whom has been discharged, and for seven professional contacts at HC Kisangani, one of whom has had treatment discontinued. Eight high-risk contacts in Kongakonga continue to refuse treatment and are demanding family care. ŷ Haut-Uélé: the presentation of August 4th does not include any patient care plan; patients in isolation, admissions and discharges are not documented and are listed as ND. The Isiro CTE is functional but remains insufficiently equipped (equipment, water and electricity) and infection prevention and control (IPC) training is underway; the province's first recovery was recorded there.
RESEARCH, INNOVATION & VACCINATION ŷ Ituri: Pillar meeting, allocation of recommendations from the rapid qualitative study to each pillar, initial drafting of protocols addressing delays in laboratory results and the actors involved in decision-making processes, and development of corresponding RDTs. 13 protocols approved by the ethics committee (100%), 3 studies completed out of 13 planned (23.0%), 3 reports produced out of 11 (30%), and 1 publication; 4 recommendations implemented out of 22 (22.7%). No documented decisions are yet based on research results, and no vaccination data are reported.	Dignified and secure burials (DSM) ŷ Ituri: 83 death alerts, 75 DHS (Death Health Surveys) conducted, including 14 in Ebola Treatment Centers (ETCs) and 63 bodies swabbed; 15 out of 32 health zones reported (46.8%). The cumulative total has reached 2,854 DHS for 3,507 alerts since May 20. Community resistance persists in Komanda (family refusal). Second day of training for two additional teams in Lita, session to identify health zones and areas where ECUMER teams will be trained and to harmonize DHS data management with all partners. ŷ North Kivu: 45 death alerts, 43 bodies swabbed and secured, 48 death assessments carried out, and 2 failures due to rushed burials in Butembo; 14 bodies remain swabbed and secured pending processing, and 11 are postponed. Deaths occurring in emergency situations predominate (23, or 51.1%), followed by community deaths (19, or 42.2%) and those occurring in Ebola Treatment Centers/ Centers (3, or 6.6%); Beni (18) and Katwa (17) account for 77.8% of the alerts. ŷ Haut-Uélé: the two deaths validated among the day's alerts were confirmed by swab in Isiro and one death

	Community data has been recorded. The EDS pillar does not include a summary table in the presentation of August 4th. ÿ South Kivu: no community deaths or post-mortem swab were reported during the day; 17 suspects are being treated at CT (Katana 8, Kadutu 5 and Miti-Murhesa 4) and 44 contacts remain in cantonment.
PREVENTION OF SEXUAL EXPLOITATION AND ABUSE (PSEA / PRSEAH) ÿ Continuation of briefing and PSEA awareness-raising.	

7 Key challenges, impacts and actions required

Challenge No.	Impact	Actions required
1	Reluctance to participate in post-mortem sampling (EDS/swabs), community resistance, and insecurity targeting EDS teams (bridge attack) Muchanga, paralysis of activities in Nyankunde)	Confirmation delays, underestimation of cases, disruption of key activities Intensify CREC, involve religious and community leaders, secure the teams, increase the number of EDS teams
2	Non-payment of service providers (strike in the health zones of Bunia and Rwampara; resumption announced after the intervention of the military governor of Ituri)	Interruption of response activities, including dignified and safe burials Regularize payments to service providers; secure dedicated funding
3	Insufficient contact tracing performance: Ituri 77.4%, North Kivu 74.1%, Haut-Uélé 49.9% and Sud-Kivu 100%, with Tshopo not reporting; the Bambu health zones, of Damascus, Kambala, Kilo and Lolwa (Ituri) remained without any listed contacts despite having a cumulative number of cases.	Uninterrupted transmission chains Train, deploy and motivate community liaisons (RECOs); strengthen supervision; consolidate the recovery observed in Ituri
4	Insufficient number of ambulances and pre-positioning at PoC	Delays in transfer and isolation Preposition ambulances (including those at PoE/PoC), deploy CTs per ZS
5	Insufficient PCI inputs (PPE, chlorine) and limited functional triage units (182/1170, i.e. 15%)	pupil infections Nosocomial risks (140 cases of PPL including 43 deaths in Ituri; 14 PPL including 3 deaths at North Kivu and 3 health workers among the confirmed cases in Haut-Uélé) Urgent supply, strengthening of IPC training
6	Geographic extension (Haut-Uélé, Tshopo) and high mobility	Risk of urban, riverine and cross-border spread Activate the 23 priority PoE/PoC points in Haut-Uélé; strengthen cross-border coordination
7	Installation of local laboratories (Rwampara, Fataki, Bambu-mine, Boga and Mambasa)	Long confirmation delays, transmission not detected Accelerate the deployment of decentralized diagnostic capabilities

Challenge No.	Impact	Actions required
8	Financial shortcomings of the resources	Limitation of the implementation of the pillars
9	Insecurity and limited access (CODECO, ADF)	Restriction of operations
10	Continuity of essential services (CEHS) compromised	Increase in indirect (non-Ebola) mortality
		Urgent mobilization of resources, advocacy with partners
		Strengthening security coordination and humanitarian access
		Effective implementation of free healthcare, strengthening of CEHS

9 Photos of field activities

Inauguration of the Rwankole CTE by the military governor of Ituri in the presence of partners.



FOR ANY INFORMATION

**ADDITIONAL INFORMATION, PLEASE
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